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Platinum Opinion

The Importance of Precise Scientific Communication: Nonrandomized Trials Should Not Be Described as Phase 3, and Registrational-intent Trials Should Not Be Described as Phase 2 617

B.A. Inman, A.J. Vickers

Standardized nomenclature for clinical trials is required so that the terms phase 1 (first-in-human dose-finding trials), phase 2 (trials to determine the safety and efficacy of a drug and the value of further research), and phase 3 (randomized trials against a standard of care) reflect the actual study design in question.

Pelvic Lymph Node Dissection in Prostate Cancer: Evidence and Implications 619

M.J. Roberts, G. Gandaglia, D.E. Oprea-Lager, J. Stranne, P. Cornford, D. Tilki, EAU-EANM-ESTRO-ESUR-ISUP-SIOG Guidelines Panel

The role of extended pelvic lymph-node dissection (ePLND) in prostate cancer has been revisited in light of evidence from studies on staging via prostate-specific membrane antigen positron emission tomography/computed tomography. It is difficult to predict which individuals might benefit from ePLND. Patients should be counselled about this uncertainty and the risks of ePLND-associated morbidity as part of the shared decision-making process.

Brief report

Magnetic Resonance Imaging Versus Computed Tomography Guidance for Stereotactic Body Radiotherapy in Prostate Cancer: 2-year Outcomes from the MIRAGE Randomized Clinical Trial 622

A.U. Kishan, J.M. Lamb, H. Wilhalme, M. Casado, N. Chong, L. Zello, J.E. Juarez, T. Jiang, B.K. Neilsen, D.A. Low, Y. Yang, J. Neylon, V. Basehart, T. Martin Ma, L.F. Valle, M. Cao, M.L. Steinberg

In the context of stereotactic body radiotherapy for prostate cancer, an aggressive reduction in margin size with magnetic resonance imaging guidance is associated with lower incidence of physician-scored genitourinary and gastrointestinal toxicity and of deterioration in patient-reported bowel and sexual function up to 2 yr after treatment.

Platinum priority papers

Heterogeneity of the Treatment Effect with PARP Inhibitors in Metastatic Castration-resistant Prostate Cancer: A Living Interactive Systematic Review and Meta-analysis 626

S.A.A. Naqvi, I.B. Riaz, A. Bibi, M.A. Khan, M. Imran, K.Z.R. Khakwani, A. Raina, M.U. Anjum, E.K. Cobran, J.L. Warner, S.A. Hussain, P. Singh, D.S. Childs, S.C. Baca, J.J. Orme, J. Mateo, N. Agarwal, S. Gillessen, M.H. Murad, O. Sartor, A.H. Bryce

For patients with metastatic castration-resistant prostate cancer, the greatest benefit from PARP inhibitor therapy is expected for individuals harboring *BRCA* alterations, with a strong signal of benefit for patients with *PALB2* and *CDK12* alterations. Patients with *ATM* or *CHEK2* alterations are the least likely to experience a benefit.

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At 5 yr after neoadjuvant androgen deprivation therapy and an androgen receptor pathway inhibitor before radical prostatectomy for high-risk prostate cancer, the 5-yr metastasis-free survival (MFS) rate was 83% overall and was significantly higher for patients with one high-risk factor (86%) than for those with two or three factors (67%). The residual cancer burden (RCB) at surgery was highly prognostic for MFS, with 5-yr MFS rates ranging from 100% to 63% for the lowest to highest RCB categories.

When Less Is More: The Challenges of Defining Residual Disease Following Neoadjuvant Hormonal Therapy in Prostate Cancer 651

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Review

A Comprehensive Systematic Review and Meta-analysis of the Role of Prostate-specific Membrane Antigen Positron Emission Tomography for Prostate Cancer Diagnosis and Primary Staging before Definitive Treatment EU★ACME 654

E. Mazzone, D. Cannoletta, L. Quarta, D.C. Chen, A. Thomson, F. Barletta, A. Stabile, D. Moon, R. Eapen, N. Lawrentschuk, F. Montorsi, S. Siva, M.S. Hofman, A. Chiti, D.G. Murphy, A. Briganti, M.L. Perera

Our study corroborates the role of prostate-specific membrane antigen (PSMA) positron emission tomography (PET) as an important tool in the diagnosis and staging of prostate cancer (PCa), offering good accuracy in detecting clinically significant PCa and metastatic disease, especially when combined with magnetic resonance imaging. Its effectiveness is notably high for high-risk patients, helping tailor treatment strategies with precise staging information. The ability of PSMA PET to detect lymph node and distant metastases underscores its importance in primary staging, guiding clinicians toward more individualized and effective treatment decisions. These findings support the integration of PSMA PET into routine PCa management, improving diagnostic precision and staging accuracy.

Full length article

Final Overall Survival and Molecular Data Associated with Clinical Outcomes in Patients Receiving Ipatasertib and Abiraterone in the Phase 3 IPATential150 Trial 672

J.S. de Bono, M. He, Z. Shi, M. Nowicka, S. Bracarda, C.N. Sternberg, K.N. Chi, D. Olmos, S. Sandhu, C. Massard, N. Matsubara, G. Chen, N.S. Bienz, D. Canter, M. Wongchenko, C. Sweeney

We investigated whether addition of ipatasertib to abiraterone and prednisone impacted survival for men with metastatic castration-resistant prostate cancer. Ipatasertib did not improve survival, but exploratory biomarker analyses for the PTEN pathway and other genomic alterations identified biomarkers of potential clinical relevance for agents targeting this pathway.

EAU Guidelines

European Association of Urology Guidelines on Renal Cell Carcinoma: The 2025 Update EU★ACME 683

A. Bex, Y.A. Ghanem, L. Albiges, S. Bonn, R. Campi, U. Capitanio, S. Dabestani, M. Hora, T. Klatte, T. Kuusk, L. Lund, L. Marconi, C. Palumbo, G. Pignot, T. Powles, N. Schouten, M. Tran, A. Volpe, J. Bedke

This overview of the 2025 European Association of Urology guidelines on renal cell carcinoma offers valuable insights into risk factors, diagnosis, classification, treatment, and follow-up, with recommendations designed for effective integration into clinical practice.

European Association of Urology Guidelines on Upper Urinary Tract Urothelial Carcinoma: Summary of the 2025 Update EU★ACME 697

A. Masson-Lecomte, A. Birtle, B. Pradere, O. Capoun, E. Compérat, J.L. Domínguez-Escrig, F. Liedberg, L. Makaroff, P. Mariappan, M. Moschini, B.P. Rai, B.W.G. van Rhijn, S.F. Shariat, E.J. Smith, J.Y.C. Teoh, V. Soukup, R. Wood, E.N. Xylinas, F. Soria, T. Seisen, P. Gontero

This overview of the 2025 European Association of Urology guidelines on upper urinary tract urothelial carcinoma offers valuable insights into risk factors, diagnosis, classification, treatment, and follow-up, with recommendations that are designed for effective integration in clinical practice.

Surgery in Motion

First-in-human Study of a Dual-modality Prostate-specific Membrane Antigen–targeted Probe for Preoperative Positron Emission Tomography/Computed Tomography Imaging and Intraoperative Fluorescence Imaging in Prostate Cancer 717

S. Chen, H. Xu, X. Chen, Q. Shen, X. Chen, M. Zhang, Z. Li, Z. Zhang, H. Hao, W. Yu, Y. Fan, L. Zhou, N. Zhang, J. Zhang, X. Yang, C. Shen, X. Li

The dual-modality prostate-specific membrane antigen (PSMA)-targeted probe ^{68}Ga-P3 is safe and feasible for PSMA-targeted positron emission tomography imaging and fluorescence-guided surgery, which facilitates tumor detection beyond conventional methods. This clinical application offers a novel tool for the surgical management of prostate cancer.

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Reply to Francesco Montorsi, Giorgio Gandaglia, Francesco Barletta, and Alberto Briganti's Letter to the Editor re: Andreas Josefsson, Marianne Månsson, Kimia Kohestani, et al. Performance of 4Kscore as a Reflex Test to Prostate-specific Antigen in the GÖTEBORG-2 Prostate Cancer Screening Trial. Eur Urol 2024;86:223–9 e112

A. Josefsson, S.V. Carlsson, H. Lilja, R. Godtman, J. Hugosson

Re: Angelo Territo, Luca Afferi, Mireia Musquera, et al. Robot-assisted Kidney Transplantation: The 8-year European Experience. Eur Urol. 2025;87:468–75 e113

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Reply to Roger L. Sur's Letter to the Editor re: Frédéric Panthier, Vineet Gauhar, Eugenio Ventimiglia, Jia-Lun Kwok, Etienne Xavier Keller, Olivier Traxer. Rethinking Stone-free Rates and Surgical Outcomes in Endourology: A Point of View from PEARLS Members. Eur Urol 2024;86:198–9 e114

F. Panthier, B. Somani, O. Traxer

Reply to Felix Melchior, Andreas Pircher, and Isabel Heidegger's Letter to the Editor re: Johann S. de Bono, Meng He, Chen Shi, et al. Final Overall Survival and Molecular Data Associated with Clinical Outcomes in Patients Receiving Ipatasertib and Abiraterone in the Phase 3 IPATential150 Trial. Eur Urol. 2025;87:468–75 e115
C. Sweeney, M. He, M. Wongchenko, R. Ferraldeschi, J.S. de Bono

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F. Melchior, A. Pircher, I. Heidegger

Re: Philip Cornford, Roderick C.N. van den Bergh, Erik Briers, et al. EAU-EANM-ESTRO-ESUR-ISUP-SIOG Guidelines on Prostate Cancer—2024 Update. Part I: Screening, Diagnosis, and Local Treatment with Curative Intent. Eur Urol 2024;86:148–63 e112
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Corrigendum

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